BASELINE → TREATMENT → FOLLOW-UP

Checks from preparation through the next treatment plan

These columns show the order of care, not equal time intervals. Baseline hearing tests are only if indicated; blood counts, kidney and liver tests repeat before each cycle. Any additional immunotherapy has its own monitoring plan.

Check	Before starting	Each chemotherapy cycle	Between visits	Response / course review	Completion / next plan
Blood count, kidneys and liver	Baseline FBC, renal and liver tests	Repeat before each cycle	Earlier tests if illness or dehydration develops	Review changes before continuation or dose modification	Follow unresolved abnormalities
Hydration and electrolytes	Assess renal/cardiac status and local fluid plan	Supervised pre/post-cisplatin fluids; salts as prescribed	Report poor intake, reduced urine, swelling or breathlessness	Reassess fluid and dose plan if kidneys change	Follow persistent kidney/ electrolyte problems
Nerves, hearing and symptoms	Record baseline nerve/hearing problems; audiology if indicated	Review toxicity and function before each dose	Report symptoms promptly; use urgent contact plan	Balance benefit and lasting side effects	Review persistent neuropathy, hearing or kidney problems
Vitamins and prevention medicines	Folate preparation; B12 in preceding week; clear steroid plan	Folate daily; B12 every three cycles; reconcile prevention medicines	Follow written plan; report missed doses or poor intake	Reconcile medicines if platinum stops or phase changes	Folate to 21 days after last pemetrexed; separate plan if maintenance continues
Disease assessment	Confirm diagnosis, intent and selected indication	Assess symptoms and clinical progress	Arrange extra assessment if indicated	Scans/tests according to consultant plan	Agree surveillance, surgery review or separate maintenance

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00317 v7 · official regimen

THE WHOLE TREATMENT JOURNEY

Pemetrexed + Cisplatin: treatment and checks

Pleural mesothelioma / non-squamous NSCLC · chemotherapy component

Both chemotherapy medicines are planned on Day 1 of a 21-day cycle. For pleural mesothelioma, the source describes up to 4–6 cycles. For advanced non-squamous NSCLC, it describes 4–6 combination cycles followed, when appropriate, by a separate pemetrexed maintenance plan (NCCP 00222). The selected preoperative nivolumab combination uses three cycles and a separate immunotherapy plan (NCCP 00849). Your team confirms which course applies and may stop or change it earlier.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	Each chemotherapy cycle	Between visits	Response / course review	Completion / next plan
Chemotherapy course NCCP 00317	Confirm disease and course	Day1 pemetrexed then cisplatin;21-day cycle	Recovery, daily folate and symptom reporting	Mesothelioma4–6; advancedNSCLC4–6; selected neoadjuvant3	Separate maintenance/surgery/ follow-up plan
Blood count, kidneys and liver NCCP 00317	Baseline FBC, renal and liver tests	Repeat before each cycle	Earlier tests if illness or dehydration develops	Review changes before continuation or dose modification	Follow unresolved abnormalities
Hydration and electrolytes NCCP 00317	Assess renal/cardiac status and local fluid plan	Supervised pre/post-cisplatin fluids; salts as prescribed	Report poor intake, reduced urine, swelling or breathlessness	Reassess fluid and dose plan if kidneys change	Follow persistent kidney/ electrolyte problems
Nerves, hearing and symptoms NCCP 00317	Record baseline nerve/hearing problems; audiology if indicated	Review toxicity and function before each dose	Report symptoms promptly; use urgent contact plan	Balance benefit and lasting side effects	Review persistent neuropathy, hearing or kidney problems
Vitamins and prevention medicines NCCP 00317	Folate preparation; B12 in preceding week; clear steroid plan	Folate daily; B12 every three cycles; reconcile prevention medicines	Follow written plan; report missed doses or poor intake	Reconcile medicines if platinum stops or phase changes	Folate to 21 days after last pemetrexed; separate plan if maintenance continues
Disease assessment NCCP 00317	Confirm diagnosis, intent and selected indication	Assess symptoms and clinical progress	Arrange extra assessment if indicated	Scans/tests according to consultant plan	Agree surveillance, surgery review or separate maintenance

What each check is for

1 Before first pemetrexed

In the week before treatment

Vitamin preparation reduces serious toxicity.

Ask / act: Confirm preparation and a single reconciled steroid prescription.

Sources: NCCP 00317

2 Before every cycle

Before dose authorisation

Counts, organ function and toxicity can change.

Ask / act: Review results before deciding continue, delay or modify.

Sources: NCCP 00317

3 At the end of combination treatment

At the indication-specific course review

Maintenance and preoperative plans differ.

Ask / act: Confirm the next regimen and which supportive medicines continue.

Sources: NCCP 00317

4 After last pemetrexed

Through the following21days

Vitamin protection extends beyond the final infusion.

Ask / act: Confirm stop/review dates with the team; request follow-up for persistent symptoms.

Sources: NCCP 00317

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-02 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.28